

Diabetes: pregnancy and rapid HbA1c change

Clinical M3-M5 | Focused Type B notes

Pregnancy changes the monitoring risk

Pre-existing diabetes versus gestational diabetes

Existing retinopathy can progress during pregnancy



Original comparison of monitoring indications. Follow-up frequency depends on retinal findings and the pregnancy pathway.

Before or during pregnancy

Situation	Retinal implication
Pre-existing type 1 or 2 diabetes	Assess retina before pregnancy or early in pregnancy; monitor based on severity.
Existing retinopathy	Counsel about progression and coordinate closer surveillance.
Gestational diabetes alone	No DR-screening indication solely from gestational diabetes.

Eye treatment needs joint planning

Sight-threatening disease in pregnancy requires retina and obstetric/diabetes discussion. Do not assume that routine intravitreal drug choices carry the same benefit-risk balance during pregnancy.

References ADA: Retinopathy Standards of Care, 2026 | NICE NG 242: Diabetic retinopathy management

Good long-term control, planned retinal review

When a rapid substantial HbA1c fall is expected

- 1 Tell the ophthalmic team before intensification when retinopathy is established.
- 2 Assess the retinal baseline and agree monitoring around the change.
- 3 Treat systemic diabetes while watching for early retinal worsening; escalate new retinal disease appropriately.

Different time horizons

Time horizon	Meaning
Long term	Better glycaemic and vascular-risk control reduces progression risk.
Early after rapid improvement	Retinopathy can temporarily worsen, especially when disease is already advanced.

Avoid two extremes

Do not abandon needed glycaemic control because of retinal risk. Do not assume that an improved HbA1c removes the need for eye review; communicate a coordinated plan with the patient and prescribing team.